

BUSINESS & PRACTICE MANAGEMENT

Group Practice Clinician Agreement Builder & Onboarding Kit

A term sheet, clause guide, onboarding checklist and departure protocol for practices that bring clinicians in, and for clinicians deciding whether to join.

AUDIENCE
Owners · Joining clinicians

FORMAT
Term sheet + guide

VERSION
1.0 · Sept 2026

TIME TO ADAPT
1 to 2 hours

WHAT IT IS

Not a contract. It is the checklist of decisions a clinician agreement must settle, with options for each, so that you arrive at your attorney with a completed term sheet rather than a blank page. It also covers onboarding a clinician and, just as important, how to handle a departure.

USE IT WHEN

You are adding your first clinician, joining a group, moving between groups, or reviewing an agreement you signed a while ago.

CUSTOMIZE FIRST

Everything. The model (employee, contractor or space-share), the pay structure, the state rules on restrictive covenants, and the records and departure terms.

HAVE AN ATTORNEY DRAFT THE FINAL AGREEMENT

Worker classification, payer contracting, records custody and restrictive covenants are governed by state and federal law that differs by state and changes. A term sheet fixes the business deal. It does not replace a contract prepared by an attorney who knows your state and your license type.

Part A: Choose the model first

Most disputes in group practices come from a mismatch between what the parties call the arrangement and how it actually works. Decide the model, then make the daily reality match the paper.

	Employee (W-2)	Independent contractor (1099)	Space-share / rent
Who controls the work	Practice sets schedule, methods, standards and caseload rules.	Clinician controls how and when they work. Practice sets only results and legal or ethical requirements.	Clinician runs their own practice. Practice provides space only.
Who bills	The practice, under its own contracts.	Varies: the practice under its contracts, or the clinician under their own.	The clinician, under their own contracts.
Typical pay	Salary, hourly or per-session rate, sometimes plus bonus.	A percentage of collected fees, or a per-session rate.	Fixed rent or a fee for use of space and services.

	Employee (W-2)	Independent contractor (1099)	Space-share / rent
Who owns the patient relationship	Usually the practice, subject to ethical duties and patient choice.	Depends on the agreement and state law. Often shared or the clinician's.	The clinician.
Benefits and taxes	Practice withholds tax and may owe benefits and payroll taxes.	Clinician pays their own taxes and benefits.	Clinician pays their own.
Main risk	Payroll cost and employment law obligations.	Misclassification if the practice controls too much.	Blurred lines if the practice actually directs the work.

CLASSIFICATION IS DECIDED BY FACTS, NOT BY THE LABEL

The IRS looks at behavioral control, financial control and the relationship of the parties, and many states use stricter tests. A practice that sets a clinician's hours, requires its notes template and its fee schedule, assigns its patients and forbids other work may have an employee whatever the contract says. Consequences can include back taxes, penalties and benefit claims. Some states also restrict ownership of psychology or medical practices by non-licensed persons (sometimes called corporate practice rules), which can affect how a group must be structured.

Part B: Term sheet (complete before drafting)

Practice (legal name)	Clinician	Start date
Model: employee, contractor or space-share	Initial term and renewal	Notice to end without cause

Term	Options and things to decide	Agreed
Role and scope	Services, populations, modalities, minimum and maximum hours, supervision or consultation needs, license and credential requirements.	
Schedule and caseload	Who sets it. Whether the practice assigns referrals. Expected availability. Caseload caps. Coverage days (see PA-01, PA-02).	
Compensation	Percentage of collections (which fees count, and net of what), per-session rate, salary, rent, or a hybrid. Payment date and statement detail. What happens with denied claims, refunds and write-offs.	
Billing and credentialing	Who bills, under whose NPI and tax ID, who enrolls the clinician with payers and how long that takes. What happens to sessions before enrollment is complete. Never bill under another clinician's NPI for services they did not personally deliver, unless the payer contract expressly allows it.	
Expenses	Who pays for office, furniture, technology, EHR, telehealth, phone, supplies, marketing, continuing education, dues and license fees.	
Malpractice insurance	Who buys it, limits, whether the clinician is named on the group policy, claims-made versus occurrence coverage, and who pays for tail coverage on departure.	

Term	Options and things to decide	Agreed
Clinical independence and standards	The clinician's independent judgment. Documentation, safety, mandated-reporting and emergency protocols the practice requires. Right to decline a patient.	
Records	Who is the custodian of records. Where they are stored. How the clinician accesses records after leaving. Retention and destruction. (See PA-17, PA-19.)	
Patients and referrals	Whose patients are they on departure. How patient choice is protected. Who may contact patients. Referral sources belonging to each side.	
Restrictive covenants	Non-solicitation of patients and staff, non-compete, or neither. Duration, geography and scope. Whether the law in your state allows it for your license type. See the note below.	
Confidentiality and intellectual property	Practice materials, forms, marketing content, clinician-created materials, and the clinician's own professional reputation and profiles.	
Marketing and profile	Directory listings, website bio, and who controls them. What happens to them on departure.	
Leave, coverage and continuity	Time off, notice for planned leave, coverage arrangements, extended leave, and professional will requirements (PA-02, PA-03).	
Ending the relationship	Notice periods, termination for cause, immediate suspension for safety or licensing reasons, and the departure protocol in Part D.	
Disputes	Mediation first, then arbitration or courts. Governing law. Who pays legal costs.	

RESTRICTIVE COVENANTS: THE LAW IS STATE BY STATE

The FTC's attempted national ban on non-competes was struck down in 2024 and the agency dropped its appeal in September 2025, so non-competes are governed by state law. Some states ban or limit them, some limit them for health professionals, and others enforce reasonable ones. Separately, patients have the right to choose their own clinician, and a clinician has ethical duties to avoid abandonment and to protect continuity of care. Many practices therefore prefer a narrow non-solicitation clause and a fair, neutral notice process (Part D) to a broad non-compete.

Part C: Onboarding checklist

Start well before the first patient. Payer enrollment commonly takes two to four months, so begin the applications as early as the license and insurance paperwork allow.

- ☐ License verified in every state where the clinician will treat, and any compact or telehealth authority confirmed (PA-10).
- ☐ Malpractice certificate received and the policy dates and limits recorded. Tail terms agreed.
- ☐ NPI confirmed, CAQH profile created or updated, and payer applications submitted with the group's tax ID. Expected start dates tracked.
- ☐ Tax and payment paperwork completed (W-9 or W-4 as appropriate), banking details entered securely.
- ☐ Signed agreement, and where relevant a business associate or confidentiality agreement.

- ☐ HIPAA and security training completed, policies acknowledged, EHR and email accounts set up with multi-factor authentication (PA-18).
- ☐ Informed consent, financial policy and privacy forms loaded in the clinician's templates (PA-09, PA-15, PA-17).
- ☐ Emergency, suicide-risk and duty-to-protect protocols shared (PA-13), and after-hours coverage arrangements set up.
- ☐ A named coverage colleague and a consultation group or peer partner assigned (PA-01, PA-04). Supervision arranged if needed (PA-06).
- ☐ Entered in the compliance calendar for license, insurance and credential renewals (PA-19).
- ☐ Introduced to the team. Practice tour, phone and telehealth setup, and first-week check-in booked.

Part D: When a clinician leaves

Departures are where good practices make or break their reputation, and where patients can be hurt if handled badly. The goals are continuity of care, patient choice, honest communication and clean records.

When	Action
Notice given	Meet within a week. Agree the last day, a transition plan and who will say what to patients. Review the agreement's notice period, patient-choice, records and restrictive-covenant terms. Consult your attorney before any dispute escalates.
Within two weeks	Identify patients by risk and need. Prioritize those in crisis, on medication, in active treatment for trauma, or with upcoming court dates. Plan warm handoffs (PA-07).
Patient notification	Send the joint notice below, or arrange it in person, as soon as the date is firm. Use neutral wording. Do not disparage either side or steer patients.
Records	Obtain patient authorizations to transfer records where needed (PA-17). Confirm who is the custodian, where records will be kept and for how long. Document any records provided to the departing clinician.
Last month	Update payer records, remove or update directory profiles and website bios, transfer voicemail and scheduling, and confirm malpractice and tail coverage.
After departure	Handle any requests for records from patients and new clinicians promptly. Keep a departure file with notices, choices made and dates.

Joint notice to patients (template)

Dear [Patient first name],

[Clinician name] will be leaving [Practice name] on [date] to [continue practice at another location / take on other work]. We wanted you to hear this from us directly, and to make sure your care continues without interruption.

You have a choice. You may (1) continue care with another clinician at [Practice name], (2) choose to continue with [Clinician name] at their new practice, [contact details, if the clinician wishes to share them], or (3) choose another provider. We will support whichever you choose. If you would like help finding a colleague, we can suggest options.

Your records. Your records belong to your care and remain confidential. If you want them sent to another clinician, you can sign an authorization form and we will send them promptly. Please call [phone] or reply to this message.

Your next appointment. Please let us know your choice by [date]. Appointments through [last date] will be with [Clinician name]. If you need help before then, call [phone]. In an emergency, call 911 or 988.

Thank you for your trust, [signatures]

IN PSYALLIANCE

Join the Town Hall channel for practice management to compare compensation models and departure protocols with other group owners. Use the Planner to organize coverage and transfers when a clinician leaves, and the Network to find colleagues who will take referrals.

Sources and further reading

- IRS, Independent Contractor (Self-Employed) or Employee? Common-law rules: behavioral control, financial control, relationship of the parties (irs.gov). Form SS-8.
- FTC, Non-Compete Clause Rule and the agency's September 2025 decision to dismiss its appeals (ftc.gov). State law on non-competes varies. Check your state.
- APA Ethics Code, Standards 3.12 (interruption of psychological services), 6.01 (documentation and records) and 10.09 (interruption of therapy) (apa.org/ethics/code).
- CAQH (caqh.org) for credentialing profiles. Your state psychology or medical board on corporate practice and record custody.

ABOUT THIS DOCUMENT

Prepared by PsyAlliance from publicly available professional and regulatory sources, reviewed against them as of September 2026. It is an educational template and practice aid. It is not legal, ethical, clinical or billing advice, and it does not create a professional relationship. Laws, licensing-board rules and payer requirements vary by state and change over time. Verify against your licensing board, professional association, payer contracts and, where the stakes warrant it, a qualified attorney before relying on it.

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